

D8 – Office Visit Note

P.A. Kim, MD | Family Medicine | 15 Sep 2011

Patient: Caduceus, Opus J. MRN: 004-7703 DOB: 14 Mar 1956 Allergies: NKDA

OFFICE VISIT NOTE

Patient: Opus J. Caduceus | MRN: 004-7703

Date: 09/15/2011 | Provider: P.A. Kim, MD

Visit Type: Urgent Visit

Practice: Oak Harbor Family Medicine (TRICARE), Oak Harbor WA

CHIEF COMPLAINT: Palpitations x 3 weeks.

HISTORY OF PRESENT ILLNESS:

55-year-old active duty USN with intermittent palpitations over the past 3 weeks. Worse at rest, particularly in the evenings. One episode of mild lightheadedness without syncope. Denies chest pain, dyspnea on exertion, fevers, or other systemic symptoms. No recent stimulant use beyond usual coffee intake. No alcohol binge. No new medications. Reports stress at work but no acute precipitant.

CURRENT MEDICATIONS:

- Omeprazole 40 mg PO daily
- Naproxen 500 mg PO BID PRN
- Flunisolide nasal spray PRN

PHYSICAL EXAMINATION:

Vitals: HR 92 (irregular) | BP 148/90 | RR 16 | Wt 88 kg

General: Mildly anxious. NAD at rest.

CV: Irregularly irregular rhythm. No murmurs, rubs, or gallops.
No JVD.

Lungs: Clear bilaterally.

Ext: No edema. Distal pulses 2+ symmetric.

IN-CLINIC STUDIES:

EKG (in clinic): Atrial fibrillation, irregularly irregular ventricular response, ventricular rate ~92, no acute ST-T changes.

ASSESSMENT & PLAN:

1. Atrial fibrillation, new onset - first episode documented today (15 Sep 2011) by EKG in clinic. No prior history of arrhythmia. Urgent cardiology referral placed to Dr. F. Mensah, Puget Sound Cardiology, Everett WA. Holter monitor ordered. Start metoprolol succinate 25 mg PO daily for rate control. Anticoagulation decision deferred to cardiology.
2. GERD - recurrent symptoms x 3 months, worsening. GI follow-up recommended.

Labs/Orders: TSH, CBC, CMP (rule out reversible causes of new A-fib)

Follow-up: Cardiology urgently. PCP RTC 4 weeks.

Electronically signed by: P.A. Kim, MD

Date: 09/15/2011 11:30